

Vaginal Hysterectomy (for Prolapse)

WARWIKI

Patient Information · Removing the uterus through the vagina, and lifting the top of the vagina

When the uterus has dropped as part of pelvic organ prolapse, one option is a **vaginal hysterectomy**. The uterus is removed through the vagina, with **no cut in the belly**. It is always paired with a **lift of the top of the vagina**, and sometimes other repairs, so the area stays supported.

About This Procedure

The uterus and cervix are freed and removed through the vagina. Removing the uterus alone does not fix prolapse. So the surgeon also **lifts and attaches the top of the vagina** to strong ligaments in your pelvis. Repairs of the front or back wall may be added, based on your exam.

Compared with surgery through a cut in the belly, the vaginal route has **no belly wound**. People often get back to normal activities sooner.

Things to Know

- Removing the uterus ends periods and the ability to get pregnant. It also removes the chance of cancer of the uterus or cervix later.
- You may be able to **keep your uterus** and have it lifted instead (a hysteropexy). Ask whether this is an option for you.
- Whether to remove the ovaries is a separate choice. Talk about it with your team.
- A procedure for **urine leakage** may be added if you need one.

Learn the Terms

Hysterectomy

Surgery to remove the uterus.

Vaginal route

Surgery done through the vagina, with no cut in the belly.

Vault suspension

Attaching the top of the vagina to strong ligaments so it stays up.

Hysteropexy

Lifting the uterus back into place without removing it.

Prolapse

When pelvic organs drop and cause a bulge or pressure.

Vaginal cuff

The closed top of the vagina after the uterus is removed.

Catheter

A thin tube that drains urine from the bladder.

Will it hurt? You will be asleep or numb during surgery. Afterward, expect mild to moderate soreness in the vagina and pelvis. Many people go home the same day or after one night.

First day

Walk soon after surgery. Your bladder emptying is checked.

First weeks

Spotting and discharge are normal. Return to activity, lifting and exercise at your own pace.

Until cleared

Nothing in the vagina and no sex until your team says so, often 6 to 12 weeks.

Follow-up visit

Your team checks healing and how well the repair holds.

How to Get Ready

- Follow the instructions about eating, drinking and blood thinners.
- Ask which repairs, and whether a leakage procedure, will be added.
- Stop smoking. Arrange a ride and some help at home.

What Happens During Surgery

- 1 You get general or spinal anesthesia, and antibiotics.
- 2 The uterus is freed and removed **through the vagina**.
- 3 The top of the vagina is **attached to ligaments**. Other planned repairs are done.
- 4 The bladder may be checked with a small camera. A catheter, and sometimes gauze in the vagina, may stay in for a short time.

Call the office that did your surgery if you have:

- A temperature of **100.4 °F (38 °C)** or higher, or chills
- Discharge that smells bad, or pain that is getting worse
- New pain in your side or back, or urine leaking from the vagina

Go to an emergency department if:

- You **cannot urinate** for several hours and your bladder feels full or painful
- Bleeding that soaks a pad in an hour, or something pushing out of the vagina with sudden pain
- You cannot reach the office and are worried

Three Things to Remember

1. Removing the uterus does not fix prolapse by itself. The top of the vagina is always lifted too.
2. You may be able to keep your uterus. Ask about a hysteropexy.
3. Walk from the first day. Wait for your team's OK before sex.